TEAM CASE STUDY: LAUREN MITCHELL

The whole class is ONE nursing team for one patient across 24 hours. We move through the time together, 4 hours at a time.

4 West, Room 4W-22 | Mon 0700 to Tue 0700

Write your team name in the header at the top of this page (it repeats on every page).

PAIR 1: MEDS & TREATMENTS	PAIR 2: VITALS & ASSESSMENT	PAIR 3: RESULTS & LABS
A name: _____	**C** name: _____	**E** name: _____
B name: _____	**D** name: _____	**F** name: _____
G FLOAT NURSE (helps wherever the charge nurse sends them): name: _____		

HOW THE TWO HOURS WORK

- The instructor moves the clock forward **4 hours at a time**: 0700, 1100, 1500, 1900, 2300, 0300, and a final check at 0700 Tuesday.
- At each checkpoint, the instructor sets the bedside monitor and reads the situation. The **charge nurse runs the page**: asks each pair the "Charge asks" question, the pair looks in the chart and at Lauren and reports. **Only the charge nurse writes** in the shared boxes while the pairs advise. The charge nurse makes one SBAR call.
- Every stop, the team also fills one column of the **response tracker** (page 3): is the treatment working? At the end you use it to **evaluate** the whole treatment.
- The packet is shared by the table; each student has their own reflection page (last page). Finish each page at the **end** of its time block. **Do not turn the page** until the instructor advances the clock.
- Use the EHR on your own device (vitals, MAR, results, notes). Your patient and the time are synced with the instructor. **The NOW vitals row:** read the bedside monitor (the instructor sets it), take your own set, do orthostatic vitals when ordered and safe, and ask the instructor for the bedside glucose. Write in dark pen.

ROLES

CHARGE NURSE (one person; changes every stop)	Runs the section. Keeps time. **Asks each pair what they found and what to do**, then decides the priorities with the team. **Makes the SBAR call and writes the team plan.**
NURSE PAIRS (the other students)	**Meds & Treatments:** MAR, orders, notes: what is running, due, held or should change, and why. **Vitals & Assessment:** the bedside monitor, orthostatic vitals, the bedside exam (mental status, skin, mouth, belly), intake and output. **Results & Labs:** new labs, ECG, trends and what they mean. Speak up when the charge nurse asks. If one of your pair is charge, your partner reports for the pair.

CHARGE NURSE ROTATION (each student is charge nurse exactly once)

Stop	0700	1100	1500	1900	2300	0300	Final 0700
Charge nurse (letter)	A	C	E	B	D	F	G
Partner reports for the pair	B	D	F	A	C	E	-
Name							

TURN-IN

One shared packet for the team, fastened with a **paperclip (no staples)**. The last page has an individual reflection and a peer check: write your own name on yours.

PATIENT CARD: LAUREN MITCHELL

Room 4W-22

Name: Lauren Mitchell, she/her, 46 years old, DOB 05/14/1980 | **MRN** SIM-40422 | 165 cm, **58.2 kg** on arrival (usual 61.0 kg)

ALLERGY: **Sulfamethoxazole-trimethoprim** (hives) | **Full Code** | Fall risk: HIGH | **Contact (enteric) precautions**: soap and water

History: **Addison disease** (autoimmune primary adrenal insufficiency) since 2021; Hashimoto hypothyroidism (2016); appendectomy at 19. Middle school science teacher.

Home meds: **Hydrocortisone** 15 mg at 0700 + 5 mg at 1400 (20 mg/day); **fludrocortisone** 0.1 mg daily; levothyroxine 88 mcg at 0600; vitamin D3 2,000 units daily.

Family: Husband **Daniel Mitchell** (48, health care proxy); son **Ethan** (15, had a "stomach bug" on Friday).

Care team: Day hospitalist Dr. R. Delacroix (0700-1900); night hospitalist Dr. K. Osei; endocrinology Dr. M. Harrington; pharmacist T. Nakagawa; dietitian E. Lindqvist.

Normal values: Sodium 136-145 | potassium 3.5-5.1 | glucose 70-110 | creatinine 0.5-1.1 (her usual 0.8) | cortisol (morning) 5-23 | ACTH 7.2-63

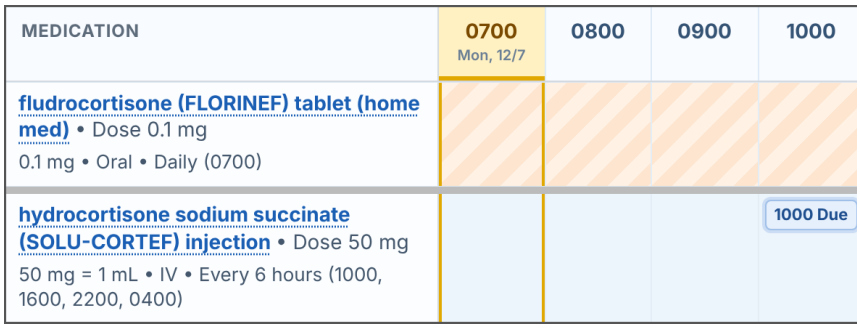
HORMONE MAP: WHY DOES SHE LOOK LIKE THIS?

fill it in at 0700

The adrenal glands make **cortisol** and **aldosterone**. In Addison disease both are missing. Write what you find in Lauren's chart and at the bedside (with numbers) for each hormone.

Hormone	What it normally does	What Lauren shows (chart numbers and bedside findings)
CORTISOL (missing)	Keeps blood vessels tight and BP up; makes glucose in the liver between meals; needed for any stress or illness; helps the kidney get rid of water.	
ALDOSTERONE (missing)	Makes the kidney keep sodium and water and get rid of potassium (and acid).	
ACTH (very high)	The pituitary pushes harder on a gland that cannot answer. ACTH comes from the same parent molecule as the skin-darkening hormone.	

FINDING IT IN THE EHR (this is what the chart shows at 0700)



MAR: the home fludrocortisone (and the home hydrocortisone, not shown) are **held**: the row is hatched. The IV hydrocortisone row shows its schedule and the next due time. Click a medicine name for the full order.

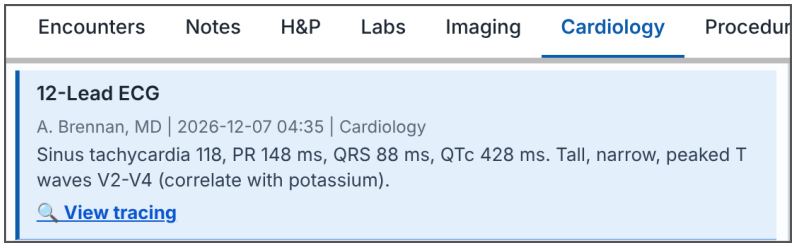


Chart Review > Cardiology: the ED 12-lead ECG. Click **View tracing** to see the strip and look at the T waves. Later ECGs appear here too.

HOW YOU EARN YOUR 20 POINTS

team grade

A complete, careful, team-minded effort earns full credit. Nothing here is a trick.

Category and points	FULL credit	GOOD (partial)	GETTING THERE
Teamwork and charge-nurse rotation **5 points**	**5.** A charge nurse is named at every stop, everyone was charge nurse once, and each pair reported its job when the charge nurse asked.	**3 to 4.** The rotation was followed most of the time. Most teammates took part.	**1 to 2.** The rotation was skipped, or one person did most of the work.
Clinical thinking at each checkpoint **6 points**	**6.** Every checkpoint has the pair boxes filled with real chart numbers and a reason (why it happens, and is the treatment working?).	**4 to 5.** Most checkpoints are filled in with numbers. One or two are thin.	**1 to 3.** Many boxes are blank or have one-word answers.
Safety, priorities and SBAR **5 points**	**5.** At 1100, 1500, 1900, 2300 and 0300: three priorities in order, a complete SBAR (all four parts) and a next step that keeps Lauren safe.	**3 to 4.** Most stops have priorities and an SBAR. A part is sometimes missing.	**1 to 2.** Priorities or SBAR are missing at many stops.
Completeness and reflection **4 points**	**4.** All boxes tried, the response tracker is filled at every stop, the final evaluation is done, and each person handed in a reflection.	**2 to 3.** Most boxes tried. The tracker, the evaluation or a reflection is missing.	**1.** Many blanks. Little reflection.

RESPONSE TRACKER: IS THE TREATMENT WORKING?

one column per stop

At every stop the charge nurse asks each pair for its numbers and writes them in that stop's column. Add an arrow (up, down, same) when it helps. In the last row circle **B**etter, **S**ame or **W**orse.

Measure	Goal	Mon 0700	Mon 1100	Mon 1500	Mon 1900	Mon 2300	Tue 0300	Tue 0700
BP lying (MAP)	SBP 100 or more; MAP 65 or more							
BP and HR standing	Negative: SBP drop under 20, HR rise under 30, no dizziness							
Heart rate / T waves	Under 100; T waves normal							
Glucose (bedside)	70-180 without dextrose pushes							
Sodium (latest)	Rising, but no more than 8 in 24 h (from 126 at 0406)							
Potassium (latest)	3.5-5.1							
Mental status	Alert, oriented x4 (her baseline)							
Urine output	0.5 mL/kg/h or more (about 30 mL/h); tell if over 250 mL/h							
Nausea / eating	No nausea; eating and drinking							
OVERALL	Better, same or worse than the last stop?	B S W	B S W	B S W	B S W	B S W	B S W	B S W

0700 | Monday

Checkpoint 1 of 6

Charge nurse for this stop: _____ **Partner reporting for their pair:** _____ ☐ everyone heard ☐ SBAR done

SITUATION It is **Monday 0700** on 4 West, room 4W-22. Lauren Mitchell, 46, has just come up from the ED. She has **Addison disease** and has been vomiting and having watery diarrhea since Saturday; she could not keep her pills down.

EMS found her confused on the bathroom floor at 0318 (BP 74/40, glucose 58). EMS and the ED gave her 2.5 L of saline; the ED also gave dextrose, **hydrocortisone 100 mg IV at 0420** and started D5NS. Her husband Daniel is at the bedside.

Time	Temp F	HR	BP lying (MAP)	Standing BP, HR	RR	SpO2	Pain	Rhythm on the monitor	Glucose
0620 prev	99.4	104	96/58 (71)	not safe	18	98% RA	4/10	Sinus tach	88 (0645)
0700 NOW									

MEDS & TREATMENTS pair reports. *Charge asks: "Which steroid is due next, and when? Why are the home hydrocortisone and fludrocortisone held? What is running in the IV?"*

VITALS & ASSESSMENT pair reports. *Charge asks: "What do the monitor and Lauren show? Skin color, mouth, mental status, belly. Is she safe to stand?"*

RESULTS & LABS pair reports. *Charge asks: "ED sodium, potassium, glucose, creatinine: what does each mean? What do the ECG T waves show? What is still pending?"*

RESPONSE TRACKER (page 3): Mon 0700 column filled ☐ Hormone map (p. 2) done ☐ | Since the last stop she is: ☐ better ☐ same ☐ worse

What changed since the last stop?

What we will do next

Our top 3 priorities *the charge nurse decides, in order*

1.
2.
3.

What we will tell the provider (SBAR) *The charge nurse makes the call.*

S Situation	
B Background	
A Assessment	
R Recommend	

Prediction for the next 4 hours *what do we think will happen?*

Charge nurse notes *ask each pair for one thing, then decide*

Turn in with a paperclip (no staples). Write in dark pen. Do not turn the page until the instructor advances the clock.

1100 | Monday

Checkpoint 2 of 6

Charge nurse for this stop: _____ **Partner reporting for their pair:** _____ ☐ everyone heard ☐ SBAR done

SITUATION It is **1100**. The 0900 labs are back, and the bedside glucose at 0900 was 72. The hydrocortisone was due at 1000. She had a large liquid stool at 0945 and the sample went to the lab.

You walk in for the 1100 glucose check. Lauren is sweating and says, "Something's wrong."

Time	Temp F	HR	BP lying (MAP)	Standing BP, HR	RR	SpO2	Pain	Rhythm on the monitor	Glucose
0700 prev	99.4	104	98/60 (73)	not safe to stand	18	98% RA	4/10	Sinus tach, tall peaked T waves	88 (0645)
1100 NOW									

MEDS & TREATMENTS pair reports. *Charge asks: "Was the 1000 hydrocortisone given on time? What does the protocol say for a low glucose in a nauseated, foggy patient: which route, how much, when do we recheck?"*

VITALS & ASSESSMENT pair reports. *Charge asks: "What is the glucose now and what symptoms do you see? Mental status compared with 0700? Urine and stool since 0700?"*

RESULTS & LABS pair reports. *Charge asks: "0900 labs against the ED labs: sodium, potassium, creatinine, glucose, lactate. Better or worse? Why can glucose fall even with D5NS running?"*

RESPONSE TRACKER (page 3): Mon 1100 column filled ☐

Since the last stop she is: ☐ better ☐ same ☐ worse

What changed since the last stop?

What we will do next

Our top 3 priorities *the charge nurse decides, in order*

1.
2.
3.

What we will tell the provider (SBAR) *The charge nurse makes the call.*

S Situation	
B Background	
A Assessment	
R Recommend	

Prediction for the next 4 hours *what do we think will happen?*

Charge nurse notes *ask each pair for one thing, then decide*

Turn in with a paperclip (no staples). Write in dark pen. Do not turn the page until the instructor advances the clock.

1500 | Monday

Checkpoint 3 of 6

Charge nurse for this stop: _____ **Partner reporting for their pair:** _____ ☐ everyone heard ☐ SBAR done

SITUATION It is **1500**. Lauren has kept clear liquids down since 1345. Endocrinology saw her. The **cortisol and ACTH** drawn in the ED before the steroid are back, as are the 1300 labs and a repeat ECG.

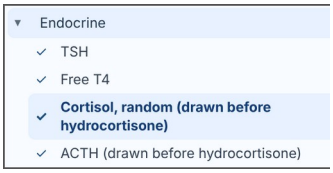
The urine hat keeps filling with pale urine. Her lying BP is over 100 and she is alert, so **orthostatic vital signs are due**.

Time	Temp F	HR	BP lying (MAP)	Standing BP, HR	RR	SpO2	Pain	Rhythm on the monitor	Glucose
1100 prev	99.0	112	94/58 (70)	not done	20	98% RA	3/10	Sinus tach (T waves a little less tall)	58 (142 after D50)
1500 NOW									

MEDS & TREATMENTS pair reports. *Charge asks: "Hydrocortisone is due at 1600 and her BP is better: give it or hold it? Why? Which IV fluid is running and at what rate? What change would you ask for?"*

VITALS & ASSESSMENT pair reports. *Charge asks: "Orthostatic vitals, done safely: lying, then standing at 1 minute. Positive or negative? Urine per hour since 1100? Mental status?"*

RESULTS & LABS pair reports. *Charge asks: "Sodium: 126 at 0406 to ___ at 1300 = ___ in ___ hours. The limit is 8 in 24 hours: on track or too fast? Cortisol and ACTH: what do they mean together? 1330 ECG T waves?"*



Lab Results > Endocrine (click a name to open it)

RESPONSE TRACKER (page 3): Mon 1500 column filled ☐

Since the last stop she is: ☐ better ☐ same ☐ worse

What changed since the last stop?

What we will do next

Our top 3 priorities *the charge nurse decides, in order*

1.
2.
3.

What we will tell the provider (SBAR) *The charge nurse makes the call.*

S Situation	
B Background	
A Assessment	
R Recommend	

Prediction for the next 4 hours *what do we think will happen?*

Charge nurse notes *ask each pair for one thing, then decide*

Turn in with a paperclip (no staples). Write in dark pen. Do not turn the page until the instructor advances the clock.

1900 | Monday

Checkpoint 4 of 6

Charge nurse for this stop: _____ **Partner reporting for their pair:** _____ ☐ everyone heard ☐ SBAR done

SITUATION It is **1900**, shift change: **your team now becomes the night shift**. The provider changed the IV fluid at 1530 after your call. Lauren has had ginger ale and juice, a regular diet was ordered at 1830, and she had one loose stool at 1640.

The 1700 labs and the stool GI panel are back. Orthostatic vitals were ordered again for 1900.

Time	Temp F	HR	BP lying (MAP)	Standing BP, HR	RR	SpO2	Pain	Rhythm on the monitor	Glucose
1500 prev	98.8	92	108/66 (80)	94/60, HR 114, dizzy	16	99% RA	2/10	NSR, normal T waves	118
1900 NOW									

MEDS & TREATMENTS pair reports. *Charge asks: "What fluid replaced D5NS at 1530 and why? Which doses are due tonight (times)? Is fludrocortisone still held, and why?"*

VITALS & ASSESSMENT pair reports. *Charge asks: "Repeat orthostatics: positive or negative? Mental status, eating, nausea, urine. Is she safe to walk with help?"*

RESULTS & LABS pair reports. *Charge asks: "Sodium at 1700 against 1300 and 0406: did the fluid change work? Potassium trend? What did the GI panel find, and what does it mean for precautions?"*

COLLECTED	RESULT	FLAG
2026-12-07 04:06	126 mmol/L	Low
2026-12-07 09:00	128 mmol/L	Low
2026-12-07 13:00	132 mmol/L	Low
2026-12-07 17:00	133 mmol/L	Low

Lab Results > Sodium > History / Trend

RESPONSE TRACKER (page 3): Mon 1900 column filled ☐

Since the last stop she is: ☐ better ☐ same ☐ worse

What changed since the last stop?

What we will do next

Our top 3 priorities *the charge nurse decides, in order*

1.
2.
3.

What we will tell the provider (SBAR) *The charge nurse makes the call.*

S Situation	
B Background	
A Assessment	
R Recommend	

Prediction for the next 4 hours *what do we think will happen?*

Charge nurse notes *ask each pair for one thing, then decide*

Turn in with a paperclip (no staples). Write in dark pen. Do not turn the page until the instructor advances the clock.

2300 | Monday night

Checkpoint 5 of 6

Charge nurse for this stop: _____ **Partner reporting for their pair:** _____ ☐ everyone heard ☐ SBAR done

SITUATION It is **2300**. Lauren ate 75% of her dinner at 2000. The 2100 labs are back. The hydrocortisone was due at 2200.
Her call light is on. "My mind won't stop. Who's going to take Ethan to school? Can I have something to knock me out?"

Time	Temp F	HR	BP lying (MAP)	Standing BP, HR	RR	SpO2	Pain	Rhythm on the monitor	Glucose
1900 prev	98.6	86	112/70 (84)	104/66, HR 96, no dizziness	16	99% RA	1/10	NSR	142
2300 NOW									

MEDS & TREATMENTS pair reports. *Charge asks: "Was the 2200 dose given? What can we offer for sleep, and which sleep medicines should she NOT get? Bedside glucose now: does she need insulin?"*

VITALS & ASSESSMENT pair reports. *Charge asks: "Orthostatics (every 8 hours), mood and mental status. What is in place to keep her from falling tonight?"*

RESULTS & LABS pair reports. *Charge asks: "Sodium at 2100: still within the limit? Potassium from 6.0 to now: where is it heading, and why? When is the next check?"*

RESPONSE TRACKER (page 3): Mon 2300 column filled ☐

Since the last stop she is: ☐ better ☐ same ☐ worse

What changed since the last stop?

What we will do next

Our top 3 priorities *the charge nurse decides, in order*

1.
2.
3.

What we will tell the provider (SBAR) *The charge nurse makes the call.*

S Situation	
B Background	
A Assessment	
R Recommend	

Prediction for the next 4 hours *what do we think will happen?*

Charge nurse notes *ask each pair for one thing, then decide*

Turn in with a paperclip (no staples). Write in dark pen. Do not turn the page until the instructor advances the clock.

0300 | Tuesday

Checkpoint 6 of 6

Charge nurse for this stop: _____ **Partner reporting for their pair:** _____ ☐ everyone heard ☐ SBAR done

SITUATION It is **0300 Tuesday**. At 0240 the bed alarm went off: Lauren had walked to the bathroom alone, had a soft loose stool, felt lightheaded and sat down on the toilet, then pulled the cord. You found her sitting there. **She did not fall.**

She is back in bed. The 0100 labs are back. Look at her and at both IV sites.

Time	Temp F	HR	BP lying (MAP)	Standing BP, HR	RR	SpO2	Pain	Rhythm on the monitor	Glucose
2300 prev	98.4	80	116/72 (87)	112/70, HR 88, no symptoms	16	98% RA	0/10	NSR	152
0300 NOW									

MEDS & TREATMENTS pair reports. *Charge asks: "The right forearm IV: what do you see and what do you do? How will the 0400 hydrocortisone go in? Anything needed for the potassium?"*

VITALS & ASSESSMENT pair reports. *Charge asks: "After the near-fall: neuro check, BP and HR lying then sitting, glucose, skin check. Is this the crisis coming back? Why or why not?"*

RESULTS & LABS pair reports. *Charge asks: "Potassium 6.0 then down to the 0100 value: why is it falling, and what is the next step? Magnesium and phosphorus? Sodium against the limit?"*

RESPONSE TRACKER (page 3): Tue 0300 column filled ☐

Since the last stop she is: ☐ better ☐ same ☐ worse

What changed since the last stop?

What we will do next

Our top 3 priorities *the charge nurse decides, in order*

1.
2.
3.

What we will tell the provider (SBAR) *The charge nurse makes the call.*

S Situation	
B Background	
A Assessment	
R Recommend	

Prediction for the next 4 hours *what do we think will happen?*

Charge nurse notes *ask each pair for one thing, then decide*

Turn in with a paperclip (no staples). Write in dark pen. Do not turn the page until the instructor advances the clock.

0700 | Tuesday

Final check, page 1 of 2

Charge nurse for this stop: _____ **Partner reporting for their pair:** _____ ☐ everyone heard ☐ SBAR done

SITUATION It is **Tuesday 0700**. Lauren slept from about 0330. The 0500 labs and the morning blood count are back. Endocrinology has written the plan to switch to pills, and she wants to go home today: "What do I do next time Ethan gets sick?"

Daniel is back at the bedside: "Show me the shot. I'll be the one giving it if she can't."

Time	Temp F	HR	BP lying (MAP)	Standing BP, HR	RR	SpO2	Pain	Rhythm on the monitor	Glucose
0300 prev	98.2	76	118/74 (89)	not repeated	14	98% RA	0/10	NSR (T waves a little smaller)	128
0700 NOW									

MEDS & TREATMENTS pair reports. *Charge asks: "The new steroid plan: which doses today, at what times? Why does fludrocortisone restart at 0800 today? What else is due this morning?"*

VITALS & ASSESSMENT pair reports. *Charge asks: "Lying and standing vitals, weight against arrival, mental status, eating, skin color. Is she ready to go home?"*

RESULTS & LABS pair reports. *Charge asks: "Sodium change from 0406 Monday to 0500 Tuesday: within the limit? Potassium now and next? Hemoglobin and eosinophils: what changed and why?"*

RESPONSE TRACKER (page 3): Tue 0700 column filled ☐

Since the last stop she is: ☐ better ☐ same ☐ worse

EVALUATE: DID THE TREATMENT WORK?

Use the response tracker (page 3) and the chart. Write the number at arrival (Mon 0700) and now, then decide: goal met?

Criterion	Goal	At arrival (Mon 0700)	Now (Tue 0700)	Goal met?
Blood pressure / MAP	SBP 100 or more, MAP 65 or more			☐ yes ☐ no
Orthostatic vitals	Negative			☐ yes ☐ no
Heart rate and T waves	Under 100; T waves normal			☐ yes ☐ no
Glucose	70-180 without dextrose pushes			☐ yes ☐ no
Sodium	Up, but no more than 8 in 24 h			☐ yes ☐ no
Potassium	3.5-5.1; normal T waves			☐ yes ☐ no
Mental status	Oriented x4 (baseline)			☐ yes ☐ no
Kidneys and urine	Creatinine back to baseline (0.8); urine 0.5 mL/kg/h or more			☐ yes ☐ no
Nausea / eating	No nausea, eating and drinking			☐ yes ☐ no

Our verdict on the treatment, in one or two sentences *is it effective? which numbers prove it?*

0700 | Tuesday

Final check, page 2 of 2

Going home safely. Same charge nurse. The charge nurse asks each pair for one more thing.

What changed since 0300 and since arrival?	What we will do next

Our top 3 priorities for the day team *the charge nurse decides, in order*

1.
2.
3.

Handoff to the day team (SBAR) *The charge nurse makes the call.*

S Situation	
B Background	
A Assessment	
R Recommend	

Meds pair: the steroid plan at home *today's doses and times, the next days, and why fludrocortisone is back*

Sick-day rules we will teach *when to double, when to triple, when to inject*	The emergency injection *what is in the kit, where, how, then what? who practices it?*

Medical ID, card and who needs to know	Our teach-back question for Lauren and Daniel

Is she ready to go home? What must be true first? *the Vitals and Results pairs advise*

Charge nurse notes (then turn to the individual reflection page)

Turn in with a paperclip (no staples). Write in dark pen. Do not turn the page until the instructor advances the clock.

MY REFLECTION AND PEER CHECK

individual

Hand this page in with your own name on it.

Name: _____ Team: _____ My charge stop: _____

1. Something I learned about adrenal crisis from Lauren's case (how the body, the treatment or the evaluation works)

2. Something our team did well (and who helped)

3. One thing I would do differently as a charge nurse or as a nurse

PEER TEAMWORK CHECK

Rate up to three teammates (not yourself). Mark one box for each.

Teammate	Did their share	Listened to teammates	Spoke up with good ideas	One kind, useful comment
	☐ yes ☐ mostly ☐ not yet	☐ yes ☐ mostly ☐ not yet	☐ yes ☐ mostly ☐ not yet	
	☐ yes ☐ mostly ☐ not yet	☐ yes ☐ mostly ☐ not yet	☐ yes ☐ mostly ☐ not yet	
	☐ yes ☐ mostly ☐ not yet	☐ yes ☐ mostly ☐ not yet	☐ yes ☐ mostly ☐ not yet	

Hand in your reflection with the team packet. Thank you!